

Standard Operating Procedure

US Army EMS Programs Management Division



US Army EMS Programs
Management Division
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Foreword

Combat Operations have validated the importance of sustainment training for the 68W Combat Medics through lives saved on the battlefield. Soldiers with Military Occupational Specialty (MOS) 68W, regardless of the additional skill identifier (ASI), will demonstrate medical proficiency utilizing Training Circular (TC) 8-800, Medical Education and Demonstration of Individual Competence (MEDIC) and by maintaining certification with the National Registry of Emergency Medical Technicians (NREMT).

Annual skills training and validation ensures Combat Medics are not lost to reclassification or separation due to non-compliance with AR 40-68, Clinical Quality Management, which requires Combat Medics to maintain NREMT certification to remain 68W MOS qualified (MOSQ). **Sustainment = Recertification = MOSQ.**

Leaders will ensure annual skills training and 68W continuing education are performed and documented in the Medical Operational Data System (MODS) 68W Tracking System.

US Army EMS Programs Management Division stands ready to assist Commanders and Soldiers in meeting this training challenge and ensure proficiency of the medical skills required to save lives on the battlefield.

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1-01. Mission Statement

US Army EMS Programs Management Division (US Army EMS) is the Army Surgeon General's execution agency for 68W Military Occupational Specialty (MOS) oversight and appointed as the National Registry of Emergency Medical Technicians (NREMT) regulatory agency. (Appendix A)

1-02. Vision

To be the foundation on which the Army Combat Medic remains medically proficient and technically competent.

1-03. Scope

US Army EMS is responsible to ensure effective standardized training is provided for all Combat Medics. US Army EMS provides strategic oversight of all 68W credentialing/sustainment training and NREMT certification for all Army components (COMPOs) and other Federal Agencies (Department of Defense, State Department and Department of Homeland Security). US Army EMS serves as the US Army's Emergency Medical Services (EMS) regulatory agency (State licensing agency equivalent) to the NREMT.

1-04. Duties and Responsibilities

The US Army EMS consists of the following branches: EMS Operations, Accreditation, Sustainment, Criminal Background and MSTC Program of Record. Primary duties and responsibilities include:

- Serves as the Army Medical Department (AMEDD) proponent for 68W Combat Medic sustainment and compliance with Army Regulation (AR) 40-68, Clinical Quality Management.
- Develops policies pertaining to MOS qualification requirements and constructive credit review in coordination with the AMEDD Personnel Proponent Directorate (APPD).
- Provides Commanders assistance and guidance through Organizational Inspection Program (OIP) and Staff Assistance Visits (SAV).
- Serves as the Contracting Officer Representative (COR) for testing and recertification payments to the NREMT.
- Approval authority for all 68W and Emergency Medical Technician (EMT) training sites and courses.
- Reviews and approves the awarding of NREMT Continuing Education (CE) credits.
- Conducts audits and inspections of Combat Medic sustainment training.
- Reviews and validate training documents for request for reciprocity.
- Conducts background investigations and/or inquiries.
- Provides program management to the Medical Simulation Training Center (MSTC) Program of Record (POR).

2-01. Scope

US Army EMS is the Army Surgeon General's agency and regulatory body for 68W MOS, Combat Medic and NREMT oversight and quality controller. The Operations Branch provides training and readiness metrics and statistics to the US Army Office of the Surgeon General (OTSG), MEDCOM and the AMEDD ensuring effective sustainment training and standardized doctrine throughout the Army. The Operations Branch oversees all aspects of the daily functions and responsibilities within the US Army EMS Programs Management Division.

2-02. Duties and Responsibilities

The EMS Operations Branch provides strategic oversight, Quality Assurance (QA) and Quality Control (QC), monitor training effectiveness and serves as an advisor/liaison to the Army Medical community on Combat Medic curriculum, doctrine and programs. Provides QC of all Combat Medic training, training sites, MODS 68W Tracking System and US Army EMS multimedia resources. Guidance and training on database systems utilized for tracking and reporting Combat Medic readiness. Coordinates the functions, efforts and capabilities of each US Army EMS Branch.

2-03. Quality Assurance

US Army EMS is the 68W Quality Assurance and Quality Control agent for the Army (Appendix B). All 68W MOS, EMT and Combat Medic training courses and sites must be officially accredited by US Army EMS. The EMS Operations Branch ensures the continued compliance with the accreditation standards and verifies training meets the current training guidance as established by MACOMs. Training activities are required to have written local Commanders' 68W Training Guidance. The OIPs and SAVs are conducted in accordance with AR 1-201, Army Inspection Policy.

The EMS Operations Branch will forward a notification letter to the Unit Command staff or Course Coordinator of the site 30 days prior to arrival unless a Staff Assistance Visit (SAV) is coordinated through higher Headquarters Command. This notice will include the standardized 68W OIP checklist (Available on the [68W Combat Medic Website](#)), evaluators working agenda and the formal notification.

The training site evaluations will inspect training site procedures. determine 68W sustainment training is being conducted to standard, increase communication, and discuss any 68W sustainment concerns. SAV and OIP Inspections will identify strengths and deficiencies. A written summary and checklist will provide:

- 1) Training Weaknesses - helps determine if the weakness can be amended.
- 2) Findings - deficiencies in training that does not meet the standards of a regulation or policy.

- 3) Observations - an area, based on the judgment of the inspecting state official can be improved.
- 4) Commendable - a judgment by the inspecting state official of superior performance of training and procedures.
- 5) Overall Summary (formal / informal report) - mandatory summary giving details of the overall inspection results will follow NLT 30 days after the completion of the inspection.

Corrective action will be required for any findings and/or training weaknesses. The corrective action requirement(s) will be clearly stated, with suspense date, and instructions for the unit to track and report to US Army EMS when the deficiency has been corrected.

The course coordinator will arrange for the briefing location with projector, provide time to conduct initial in brief and exit brief, invite Commanders and training leaders, provide physical address of briefing location, and a Blackberry / cell number in case of emergency.

The inspection will begin with a site management review of documentation. This includes but not limited to:

- Commander's written training guidance
- Instructor files
- Student files
- Unit SOP
- Training Schedule
- Class roster
- Sign in/out for both students and instructors
- DA Form 7442, certificate, etc.
- 68W Individual Status Review in MODS (if applicable)
- 68W MODS entries
- Risk management
- Student critiques/AARs

The classroom, equipment, class VIII, and lanes will also be inspected for authenticity, focus of education, quality of instruction, and assurance of realism.

The didactic portion of the course will be inspected as well as the skills validation of table VIII.

2-04. 68W MODS Tracking System

US Army EMS is responsible for the qualitative oversight and management of data in the 68W MODS Tracking System (commonly referred to as "MODS"). The 68W MODS secure database accesses information for every Combat Medic Army wide. 68W MODS provides Commanders real-time information on the qualifications, training, and readiness of Combat Medic personnel and organizations. The EMS Operations Branch processes requests, verifies authorization and grants the read/write access to the 68W MODS Tracking System. US Army EMS conducts random and discovery audits of 68W MODS data on a regular basis. 68W MODS users are subject to formal audit for validation and verification purposes. An audit notification does not necessarily imply fraud is suspected or input errors have been discovered. Information entered in error must be reported as soon as possible to US Army EMS Operations Branch for correction.

2-05. 68W MODS Writers

Each unit (per UIC) will have two (2) appointed personnel as 68W MODS Writers. Each 68W MODS Writer will be approved by the Unit Command or Leadership. Specific requirements for 68W MODS Writers are detailed in MODS Policy #4 (Available on the [68W Combat Medic Website](#)). The 68W MODS Writers are responsible for entering data into the 68W MODS Tracking System and maintaining the required supporting documentation. 68W MODS writers are trusted agents and must validate training prior to recording data into 68W MODS. All data entered is subject to review and audit by US Army EMS and the NREMT. 68W MODS writers must obtain the following documents for verification of training prior to entering data into 68W MODS:

1. Training Schedule
2. Sign in/out roster
3. DA 7442 (TC 8-800)
4. Training certificate (if applicable)

Partially completed training (incomplete MEDIC tables I-VII) is not authorized for recertification with NREMT. All training and tasks associated with a MEDIC Table must be complete prior to recording the completion of the Training Table in MODS. Memorandums are not accepted as proof of training.

Unauthorized use of the 68W MODS site may subject the writer to criminal prosecution. Evidence of unauthorized use collected during monitoring may be used for administrative purposes and could result in loss of NREMT certification and MOS as well as criminal prosecution.

2-06. Information Management and Database Administration

The AMEDD Regulation 10-1 appoints US Army EMS as the proponent for all matters concerning 68W MOS sustainment training and subsequent NREMT renewal. US Army EMS manages and maintains content for several webpages and network databases to support this mission. The sites provide up to date information related to all facets of 68W, Combat Medic and EMT training resources. Sites managed by the US Army EMS include:

- 68W Combat Medic Website
- 68W EMS Portal (see para 3-04b)
- 68W MODS Tracking System (see para 2-04)

2-07. 68W Combat Medic Website

The 68W Combat Medic website is the official informational platform of US Army EMS. The site is hosted on the Army Enterprise Network and accessible to personnel with a Common Access Card (CAC). The website provides access and links to: training documents, policies & references, course management resources, US Army EMS points of contact (POC) and current news and information pertaining to combat and emergency medicine. The downloads section provides access to numerous downloadable tools including: training documents, regulations, policies and sustainment training resources.

2-08. Reinstatement

In accordance with Army Regulation (AR) 40-68, Clinical Quality Management, Combat Medics (E-7 and below) must obtain and maintain certification as a NREMT EMT (or higher). ALARACT 058/2010 (Available on the [68W Combat Medic Website](#)) summarizes the requirements for sustainment and certifications required to maintain MOS qualification. It also details actions required for soldiers who fail to maintain MOS qualification. There are no extensions or waivers to the recertification period. Combat Medics with mitigating circumstances that resulted in a lapsed NREMT certification may request an exception to policy with endorsement by the first O-5 in their Chain of Command. US Army EMS Director serves as the approving authority for all requests for reinstatement of NREMT certification. A sample memorandum and reinstatement requirements can be found on the [68W Combat Medic Website](#).

2-09. Constructive Credit

Training events not previously approved by US Army EMS, requires a constructive credit analysis (also called “Crosswalk”) prior to the start of the event. The crosswalk process validates the training and ensures proper distribution of NREMT continuing education (CE) credits. All training is awarded one CE for one hour (50 minutes) of training. Constructive credit crosswalks are analyzed based on two categories:

- 1) Enduring event – Enduring events occurs on a regular basis and impact a majority of 68W such as EFMB training.

- 2) Non-enduring event – Non-enduring events are conducted infrequently for a small group of 68W such as ARNG annual seminar, Medical/EMS Seminars, Conferences, Civilian pre-hospital courses, Allied Health Courses, etc.

Crosswalk requests must be submitted to US Army EMS between 1 April and 1 October and no later than 30-days prior to start date. Requests must include the following documents:

- Course schedule that reflects hourly blocks of instruction
- Lesson plans/learning objectives
- Sample certificate of completion (with certifying authorities' signature)

Upon approval, training events will be added to the 68W MODS course list.

3-01. Scope

US Army EMS is the regulatory agency that validates and verifies 68W Combat Medic training meets the US Army and NREMT standards. Accreditation is the formal approval by US Army EMS for an organization to officially conduct approved courses.

Accreditation recognizes that the training site has met and maintains the high standards necessary for the student to establish and maintain proficiency as a 68W Combat Medic. All Combat Medic and NREMT training conducted at an Accredited Training Site must be approved by US Army EMS for QA and accountability.

3-02. Duties and Responsibilities

The US Army EMS Accreditation Branch receives, reviews and approves requests to establish an approved training site. Once approved, the training site is assigned an accreditation site code and is authorized to conduct training. The Accreditation Branch maintains oversight and QA of all approved training. US Army EMS Accreditation Branch reviews and approves requests for read/write access to the Army EMS Training Portal.

3-03. Accreditation

Accreditation brings trust, improved readiness and provides opportunities for Combat Medics to obtain and maintain medical proficiency and competence. Through accreditation and maintaining high quality standards for medical training, the Army can create an agile, adaptive, and innovative Combat Medic ready to perform in current operations and beyond. The training required to meet the US Army and NREMT standards supports the Force by enhancing operational agility, improving the Combat Medic's knowledge, and creating trusted medical professionals across the total force in support of our Nation's endeavors.

Accreditation is a two phase process that will be completed prior to starting a training event. Phase one is obtaining an Accreditation Site Code. The Accreditation Site Code certifies that the organization has met the requirements to conduct accredited training. Phase two is obtaining an Approved Course Number for each training event. The Approved Course Number certifies the curriculum and training requirements have been accredited by US Army EMS to meet the requirements for 68W sustainment and NREMT recertification. The Accreditation Site Code and Approved Course Number may be requested together if all requirements are met.

3-03a. Accreditation Site Code

Accreditation requests are submitted to US Army EMS Accreditation Branch no later than 30-days before start of first course. Once approved, US Army EMS assigns an Accreditation Site Code and issues a Memorandum of Accreditation. The Accreditation Site Code must be obtained prior to conducting courses. Students will not receive credit or CEs for training conducted at a training site that has not been accredited and approved by US Army EMS. Accreditation Site

Codes cannot be moved or transferred without US Army EMS approval. Exceptions must be approved by the Director of US Army EMS.

The accreditation requirements and process is outlined in Appendix C.

Site Code Application documents can be found in the 68W Downloads section of the [68W Combat Medic Website](#).

3-03b. Approved Course Number

Combat Medic courses or training events conducted at the accredited training site must be individually approved by US Army EMS and assigned an Approved Course Number. Without an Approved Course Number, credit will not be applied towards 68W MOS sustainment or NREMT recertification.

To initiate the approval process, Course Coordinators will submit a training schedule no later than thirty (30) days prior to the training event.

Regularly approved courses include:

1. Training Circular (TC) 8-800 (MEDIC) Tables Training (Appendix H)
2. 68W TC3 Sustainment Course
3. 081-68W10, Combat Medic Reclassification Course (MOS-T)
4. Nationally Registered Paramedic (NRP) Sustainment Course

Courses not regularly approved or not registered in the MODS requires a Crosswalk (see para 2-09).

The requirements and process to obtain an Approved Course Number is outlined in Appendix D.

3-04. Management of Accredited Training Site

Management and day to day operations of an Accredited Training Site is the responsibility of the requesting Command/Organization. US Army EMS provides oversight of Combat Medic training sites and is responsible for ensuring effective standardized training.

3-04a. Accredited Training Site Personnel

At minimum, the following Key Personnel will be appointed to manage and oversee the Accredited Training Site:

- **Medical Control Officer (MCO)** – The MCO is a licensed provider with emergency medical experience. The MCO must be an Emergency Medical Physician (62A), Physician Assistant (65D) or Emergency Room Nurse (66T). Other providers with previous Combat Medic or emergency medicine experience may serve as the MCO with approval from US Army EMS Director. The MCO is the medical authority for all training/courses conducted at the accredited training site and verifies quality of training.

The MCO will review and approve invasive procedures (involving entry into the body by cutting or by inserting an instrument) in support of training. The Medical Director/Medical Control Officer is responsible for profile decisions.

- **Course Coordinator (CC)** – The CC is a Noncommissioned Officer (NCO) or Department of Defense (DoD) Civilian with NREMT certification of EMT or higher, IAW AR 40-68. The CC oversees the training programs and serves as the liaison between the students, the accredited training site staff, the local medical community, and US Army EMS. The CC is responsible for all records keeping, equipment and maintaining the EMS related databases for their course/site.
- **Primary Instructor (PI)** – The PI is an NCO or Civilian with NREMT certification of EMT or higher. The PI is the Subject Matter Expert (SME) in all aspects of combat medicine and pre-hospital emergency care. The PI must be proficient in techniques and methodologies of adult education, classroom management and resource and personnel management.
- **Instructor** - Instructors are Officers, NCOs or Civilians with NREMT certification of EMT (with current BLS/CPR certification) or higher and/or a SME in the topic areas in which they are instructing. A SME is a person with the education, credentials, certifications and/or experience in a given field of which they are considered by their peers or superiors to be an authority or expert. Instructors conduct all aspects of didactic and practical exercise training, course preparation and classroom management. Instructors must be 68W or 18D MOS qualified (MOS-Q) and certified/credentialed in the course/subject matter in which they are teaching. Course Coordinators will verify each Instructors' Combat Medic skills prior to conducting a course. Instructors must have completed (and documented) training of TC8-800 Tables I through VII and Table VIII skills validation annually.

The MCO, CC and PI will be on Additional Duty Appointment Orders signed by their Commander (or designee). US Army EMS will be notified of any changes in personnel and provided an updated copy of the Key Personnel Roster as soon as possible. Changes in personnel may require accreditation documents be updated and re-submitted to US Army EMS.

3-04b. 68W EMS Portal

The 68W EMS Portal is a database program used to manage Combat Medic training courses and training site locations. US Army EMS utilizes the portal to record administrative data, track statistics and manage courses. Information provided includes: course types, training site/location, dates of training, numbers of seats available, POC and on-line course registration capabilities.

Course Coordinators are required to use the 68W EMS Portal to request training events. At a minimum, Course Coordinators will:

- Input and maintain site point of contact information
- Input data for requested courses NLT than 30 days prior to course
 - o Course type
 - o Start date
 - o End date
 - o Practical (skills validation) & NREMT exam date
 - o Total seats (available)
- Provide Army EMS day 1 sign in/out roster within 24 hrs
- Input course roster no later than 2-weeks prior to start date
- Update course info as soon as possible when changes are made
- Close out completed course within 24-hours
 - o Check “Completed” box under Course Info
 - o Check “Cancelled” or “Completed” for each student on course roster
- Check “Non Conduct” and enter reason in Course Note box for cancelled courses

Course Coordinators will ensure all information on the 68W EMS Portal is accurately maintained and will notify Army EMS of any changes (date changes, non-conduct, etc.) made to the course information. US Army EMS Accreditation Branch reviews and processes all requests for read/write access to input course data.

3-04c. Site Records Management

Accredited training sites will keep and maintain student and training records for a period of two (2) years. Each courses' training related documents/files will be readily available for review or inspection by US Army EMS. Information for each course will also be input into the designated US Army EMS database and kept current. Training record storage requirements is to be hard copy at the training site. At minimum, the following records will be kept for each course:

- Memorandum of Accreditation
- Key personnel roster
- Unit Standard Operating Procedures (SOP)
- Key personnel appointment orders
- Instructors' credentials
- Training schedules
- Risk Management worksheets
- Class rosters
- Daily student sign-in/out sheets

- Course completion document (DA7442, certificate, etc.)
- Course critiques or After Actions Reviews (AAR)

All training site records are subject to audit by US Army EMS. Details of required training site records can be found in Appendix E.

3-05. Suspension / Revocation of Accreditation

US Army EMS may revoke or suspend a training site's accreditation for failure to act in accordance with the policies set forth in this SOP, NREMT policies or applicable Army Regulations. Additionally, accreditation may be revoked or suspended for the following violations:

- Conducting training without properly credentialed personnel
- Conducting training without a valid Accreditation Site Code or Approved Course Number
- Training at an alternate location other than the approved site (without approval by US Army EMS)
- Falsification of any documents or information submitted to US Army EMS (includes data entered into training data bases)

4-01. Scope

US Army EMS is the regulatory agency that verifies and validates that 68W Combat Medic training meets the US Army and NREMT standards. The US Army EMS Sustainment Branch provides a valid and uniform process to assess and certify the knowledge and skills required of proficient and competent Combat Medics.

4-02. Duties and Responsibilities

The Sustainment Branch manages and assists with 68W MOS sustainment and NREMT recertification for Combat Medics in each Army COMPO:

- COMPO I - Active Army
- COMPO II - Army National Guard (ARNG)
- COMPO III – U S Army Reserve (USAR)

Utilizing the designated US Army EMS database, the Sustainment branch collects and analyzes 68W MOS and NREMT data for tracking and reporting the readiness of Combat Medics in all Commands. The Sustainment Branch reviews, audits and validates recertification credentials and sustainment training in the 68W MODS Tracking System as well as the NREMT database. Data analysis is used by the Sustainment Branch to develop strategies and make recommendations for improved cognitive and psychomotor evaluations.

4-03. 68W Combat Medic MOS Sustainment

In accordance with AR 40-68, Clinical Quality Management, Combat Medics (E-7 and below) must obtain and maintain certification as a NREMT EMT (or higher). Additionally, each Combat Medic must demonstrate their knowledge and ability to perform critical tasks and life-saving skills in accordance with STP 8-68W13-SM-TG.

4-03a. NREMT Certification

NREMT recertification is required every 2 (two) years. Every 68W must affiliate with the US Army for NREMT recertification. The NREMT recertification process is outlined in Appendix F. Regular QA reviews are performed on recertification credentials to ensure compliance with US Army regulations & policies and NREMT standards. US Army EMS may recommend denial, revocation or other action based on audit findings. It is the individual responsibility of the Combat Medic to obtain required sustainment training and complete the NREMT recertification process. US Army EMS Sustainment Branch is the liaison between the US Army and the NREMT on all matters relating to recertification of Combat Medics.

4-03b. Combat Medic Knowledge and Skills Sustainment

Each Combat Medic must sustain proficiency of combat medical skills and knowledge. Every Combat Medic is required to validate critical combat medical skills every twelve (12) months. The TC8-800, Medical Education and Demonstration of Individual Competence (MEDIC) training tables (I-VIII) provides the necessary requirements for Combat Medic skills sustainment.

4-03c. 68W as a Secondary MOS

To perform duties as a 68W Combat Medic, personnel with a secondary MOS (SMOS) of 68W must have a valid NREMT certification and current sustainment training as outlined in this SOP. Additionally, verification of attendance (in the form of a DD Form 1059) of an accredited 68W MOS producing school must be submitted to the Sustainment Branch. Recertification for personnel with a 68W SMOS differs from those with a 68W primary MOS. The SMOS recertification process is outlined in Appendix G.

4-03d. Physical Limiting Medical Profile

Combat Medics with a temporary medical profile that limits physical activity to the point that prevents participation in Training and Skills Validation (Tables I-VIII) must request deferment options from US Army EMS, if applicable. Combat Medics with a temporary or permanent medical profile that limits physical activity, but not to the point of prohibiting participation, must obtain approval from the Course Medical Director/Medical Control Officer prior to starting any training or skills validation activities. Physical performance abilities must meet guidelines as published in DA PAM 611-21.

4-04. Sustainment Records

68W sustainment training will be documented and recorded in the 68W MODS Tracking System.

Partially completed training (incomplete MEDIC tables I-VII) is not authorized for recertification with NREMT. All training and tasks associated with a MEDIC Table must be complete prior to recording the completion of the Training Table in MODS. Memorandums are not accepted as proof of training. TC8-800 is intended to be trained and tested in whole for all eight tables.

US Army Sustainment Branch tracks and verifies the completion of Combat Medic training by review of training records and audit of the 68W MODS Tracking System.

4-05. Failure to Maintain NREMT Certification

Combat Medics (E-7 and below) who fail to recertify their NREMT certification are classified as non-MOSQ and will immediately be suspended from all medical related duties and are subject to appropriate personnel action. The Sustainment Branch tracks, records and reports all personnel who fail to maintain NREMT certification and/or MOS qualifications to the US Army EMS Director. All Combat Medics will be properly counseled (to include appeals process) by their supervisors upon notification of expiration, suspension or revocation.

4-06. EMT Reciprocity

US Army EMS Sustainment Branch provides verification of certification and training for Combat Medics requesting reciprocity for State EMT certification. Requests submitted to the US Army EMS Sustainment Branch must include the following:

- Completed State reciprocity application form
- Army EMT/68W course completion certificate
- Copy of NREMT certification card
- Copy of Cardiopulmonary Resuscitation (CPR) card
- Department of Defense (DD) Form 214

5-01. Scope

US Army EMS has a duty to investigate and exclude individuals who pose a risk to public health, safety and welfare by virtue of conviction or disciplinary action for certain crimes/acts. The Criminal Background Branch (CBB) assures that Combat Medics are morally capable of the extraordinary responsibility and professionalism required to perform duties in positions of the highest trust. US Army EMS CBB safeguards against individuals who might pose a risk or danger to those who are in a state of extreme physical and emotional vulnerability.

5-02. Duties and Responsibilities

The CBB provides in-processing Criminal Background Questionnaire (CBQ) briefings for 68W MOS-T, 68W Initial Entry Training (IET), Advanced Individual Training (AIT) and initial NREMT certification courses.

The CBB reviews and investigates information provided by the applicant to determine eligibility for NREMT certifications. The CBB also receives and investigates information from other sources such as:

- Unit Leadership
- Public records
- Publications / media
- Other Federal or State agencies

CBB provides information, guidance and assistance to Commanders on issues relating to 68W MOS eligibility. CBB notifies the NREMT, Soldier's Commander, and the Individual Soldier and the Course Coordinator (as applicable) following any eligibility denial or revocation.

5-03. Criminal Background Questionnaire

All persons requesting eligibility as a Combat Medic, EMT, Advanced Emergency Medical Technician (AEMT) or NRP must complete a CBQ (Available on the [68W Combat Medic Website](#)). The CBQ provides the applicant the opportunity to declare information which may affect their NREMT eligibility. Each CBQ is verified and reviewed to identify information which may require further investigation. CBQs must be completed and forwarded to CBB within 72hrs of reporting for class.

5-04. Eligibility Considerations

The US Army EMS Director may deny or revoke a Soldier's 68W MOS eligibility when a qualifying conviction, Chapter separation proceedings or Uniform Code of Military Justice (UCMJ) disciplinary action has occurred. A detailed list of disqualifying factors are provided in EMS Policy #1 (Available on the [68W Combat Medic Website](#)). Notification for eligibility denials and revocations are sent to the NREMT, the Commander, and the Individual Soldier.

Eligibility (initial, current or recertification) determination is based on the nature and severity of the act(s) committed under either UCMJ or Civilian Jurisdictions. The US Army EMS Director is the authority for denying or requesting eligibility/certification.

5-05. Reporting Requirements

All certified health care personnel (including 68W Combat Medics) will immediately notify their Commander (or supervisor) of situations or conditions that affect their eligibility for certification or recertification. In accordance with All Army Activities (ALARACT) 100-2016 (Available on the [68W Combat Medic Website](#)), Commanders will report any pertinent information which may impact 68W MOS or NREMT eligibility to the US Army EMS CBB. Reporting is mandatory for the following actions/situations:

- Any request for revocation or suspension of NREMT certification related to scope of practice violations
- The fraudulent submission or possession of altered documentation, credentials or CE certificates for the purpose of certification or recertification
- Initiation of formal criminal charges for an offense involving sexual misconduct, domestic abuse or controlled substances
- Any initiation of formal charges under the UCMJ
- Any misdemeanor or felony charges for offenses covered under the laws of the United States, it's States and Territories
- Imposition of non-judicial punishment under authority of Article 15 of the UCMJ
- Any initiation of involuntary separation action
- Placing a Combat Medic in a status of Absent Without Leave (AWOL), Deserter, or Dropped from Rolls (DFR)
- Combat Medics with an unsolved mental/behavioral health diagnosis or hospitalization in a mental health facility within the last three (3) years involving potential harm to self or others
- Combat Medics with mental or physical condition which exist for more than six (6) months that compromises their ability to perform medical duties
- Combat Medics enrolled in an Impaired Healthcare Personnel Program or the US Army Substance Abuse Program (ASAP)

Initial reporting of information and/or requests for revocation to the US Army EMS CBB requires a formal memorandum signed by the Commander. All revocation requests are reviewed and considered by the US Army EMS Director for final disposition. Official notification of any adverse administrative action taken by US Army EMS is forwarded to NREMT, the requesting Commander and the Soldier.

5-06. Reinstatement

Formal requests (official memorandum format) for reinstatement of eligibility/certification is submitted to US Army EMS by the Commander and are considered on a case-by-case basis. A sample memorandum and reinstatement requirements can be found on the [68W Combat Medic Website](#). The US Army EMS Director is the approving authority for reinstatement requests.

5-07. Appeals

Personnel in training denied eligibility have 72-hours, from the time they are notified, to submit (to the Course Coordinator) an appeal request. The Course Coordinator will forward the appeal to the US Army EMS CBB within 24-hours of receipt of the appeal. The US Army EMS Director is the approving authority for appeals.

Combat Medics, other than 68W IET/AIT Soldiers, whose medical privileges have been revoked will submit requests for appeal directly to the NREMT Appeals Committee. Appeals must be submitted in writing within 45-days of denial notification counseling. Unit Commanders will immediately notify US Army EMS CBB of the individual's intention to appeal revocation.

Official notification of appeal decision is forwarded to the Commander, the Soldier and NREMT.

6-01. Scope

Medical Simulation Training Centers (MSTCs) are an Army training asset (Army funded program) located at installations in both the continental United States (CONUS) and outside continental United States (OCONUS) locations. The US Army EMS works in partnership with the Training and Doctrine Command (TRADOC), Installation Management Command (IMCOM), and the Program Executive Office for Simulation, Training and Instrumentation (PEOSTRI) Medical Simulation Program Manager (PM MedSim) to manage each MSTC and to ensure compliance with training policies and regulations. US Army EMS is appointed by MEDCOM to provide regulatory oversight of the US Army MSTC Program worldwide.

6-02. Duties and Responsibilities

The MSTC provides an accredited and standardized medical training platform for both classroom and simulated battlefield conditions. The MSTC is a total Army training asset providing training for Soldiers within a specified region. The MSTC provides Combat Medic sustainment, NREMT certification and recertification, Combat Life Saver (CLS) and other medical training as needed by local/regional Commanders. The MSTC program provides and supports training for medical and non-medical personnel including Active Duty, Reserve, and National Guard, with priority given to deploying units. The MSTC's primary mission is to prepare Combat Medics perform medical interventions under combat conditions. See 3-04a for further description.

6-03. Programs of Instruction

MSTCs must maintain accreditation from US Army EMS. MSTCs will operate and conduct training/courses in accordance with the guidelines and policies of the US Army EMS SOP. Courses and curriculum must be approved by US Army EMS and meet the intent and scope of the MEDCOM, FORSCOM, NGB and USARC Commander's Training Guidance.

6-03a. Scheduling of Training

MSTC courses and training events are scheduled and conducted regularly by MSTC staff. These courses are forecasted annually and training schedules posted on the 68W EMS Training Portal. Training calendars for the upcoming fiscal year (FY) will be forwarded to US Army EMS no later than the first day of September each year and as updated. Some MSTC courses are also listed on the Army Training Requirements and Resources System (ATRRS). However, courses listed on ATRRS that are not accredited by US Army EMS are not recognized for sustainment or recertification credit. Courses listed in ATRRS should be verified through the 68W EMS Training Portal.

MSTC facilities and training resources are available for individual unit use. Requests for use of the MSTC facilities (or additional training events) may be presented at the quarterly MSTC planning meeting, through the local garrison

Directorate of Plans, Training, Mobilization and Security (DPTMS) office or directly to the MSTC. Approval for use is based on the priority of training and the availability of the MSTC.

6-03b. Training Priority

The MSTC allocates training and resources according to the following priorities:

1. Priority One: Deploying Units – Units which are scheduled to deploy within six months. These units have priority over all (scheduled or unscheduled) training to meet their Division/Corps Combat Medic and CLS deployment requirements.
2. Priority Two: Sustainment – Training and validation of Combat Medic skills (TC 8-800, tables I and VIII) and NREMT recertification. CLS training and certification for non-medical personnel.
3. Priority Three: As Available – Other medical training desired by Commanders within the region. Provided only when time and resources allow. This training includes, but is not limited to: CLS recertification, continuing education and skills training, EMT courses and validation of skills for unit trainers.

6-04. MSTC Operations

As an Army training asset, the MSTCs are managed and supported by IMCOM, TRADOC and PEOSTRI. As a medical training institution, MEDCOM maintains operational oversight of the MSTC program. US Army EMS is appointed by MEDCOM to provide regulatory oversight of each MSTC.

6-04a. Training Validation

US Army EMS is the QA and QC agent for MEDCOM, the OTSG and the AMEDD for all Combat Medic training. Each MSTC will follow and comply the policies and guidelines for accreditation/validation as outlined in this SOP.

6-04b. MSTC Oversight Committee

Each MSTC has a local Oversight Committee. The Oversight Committee is the checks and balance system created to ensure local and regional Commanders' training needs are met. The Oversight Committee meets at least quarterly to discuss training events and AARs, review requests and training schedules, resolve issues and to provide a forum for local stakeholders to offer input.

The Oversight Committee is made up of the following personnel:

1. MSTC Course Coordinator (CC) - Serves as the Oversight Committee Chairperson
2. MSTC Medical Control Officer (MCO)
3. Chief, Garrison DPTMS - Represents IMCOM
4. Chief, Hospital Education/Training and/or Operations for local Medical Center (MEDCEN) or Medical Department Activity (MEDDAC)
5. Regional Brigade or Battalion Surgeons
6. Regional Battalion Level Training Officers - from units not supported by Corps/Division/Brigade

6-04c. Daily Operations

Daily operations of the MSTC are the responsibility of the following key personnel:

1. Medical Control Officer (MCO)
2. Course Coordinator (CC)
3. Site Lead (Contractor)
4. Primary Instructor (PI)
5. Assistant Instructor (AI)
6. Quality Assurance Representative (QAR)

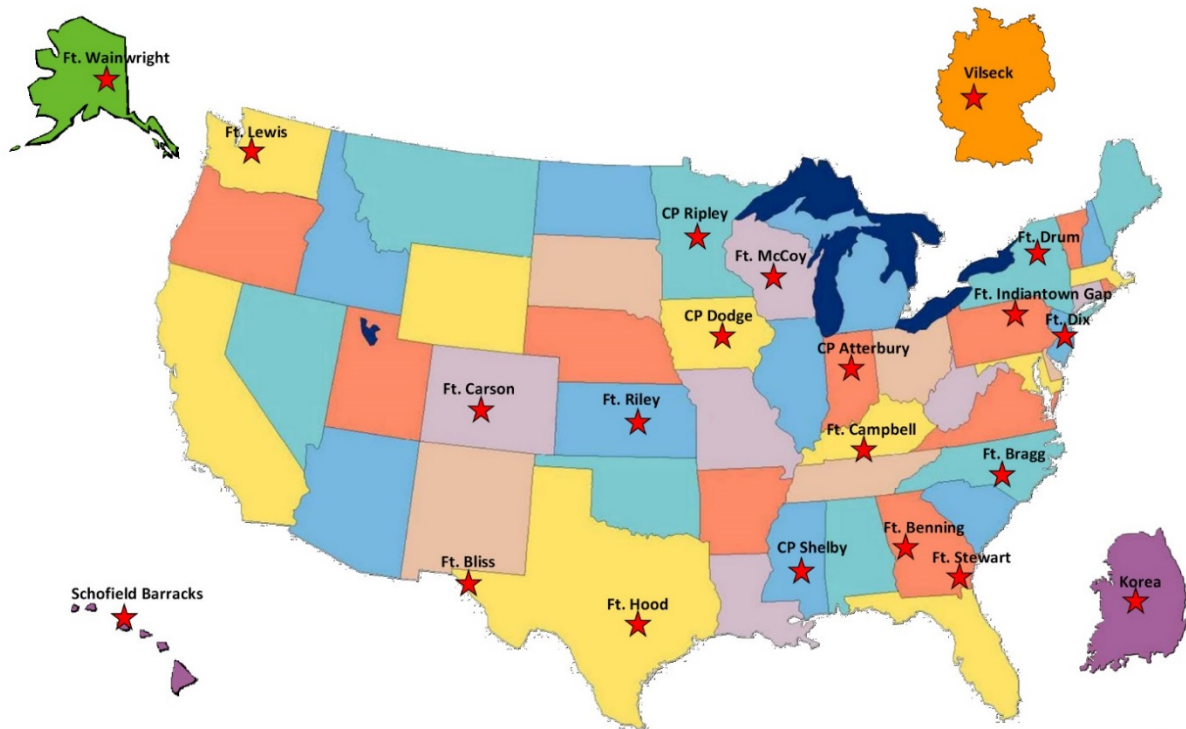
Each Key Person will be on Additional Duty Appointment Orders signed by their Commander (Military) or Supervisor (Civilian). Additional Duty Appointment Orders sample can be found the [68W Combat Medic Website](#). US Army EMS will be notified of any changes to Key Personnel and provided an updated copy of the Additional Duty Appointment Orders as soon as possible.

6-04d. Reporting/Records Keeping

MSTCs will keep and maintain student and training records as directed in Chapter 3 of this SOP. Additionally, the MSTC will maintain the following:

- Quarterly Oversight Committee meeting agenda & minutes
- Field Service Report (FSR)
- Inventory records and reports
- Course Completion File
- Quarterly and Annual reports
- Other records or documents as required by Army policy and regulation

6-05. MSTC Locations



**National Registry of
Emergency Medical Technicians®**
THE NATION'S EMS CERTIFICATION™

ROCCO V. MORANDO BUILDING
6610 BUSCH BLVD.
P.O. BOX 29233
COLUMBUS, OHIO 43229

(614) 888-4484
www.nremt.org

January 24, 2013

Mr. James M. Aplin
Chief, US Army EMS Programs Management Division
2450 Stanley Road, BLDG 146
Fort Sam Houston, TX, 78234-7517



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Dear Mr. Aplin,

Sustainment of knowledge, skills and abilities over tasks requiring competent patient represent the core to successful delivery of care in emergent situations. For this reason the National Registry of EMTs and the State licensing agencies require ongoing education for all EMS providers.

Sustainment program success requires monitoring of outcomes to assure authenticity, focus of topical education, quality of instruction, education equipment readiness and assurance that new patient care therapies are being instructed. States that fail to monitor sustainment have found fraud, repetitive and old therapy instruction, missing educational equipment and unqualified instructors. When this environment becomes normal recession of quality emergency patient care follows. Patient outcomes in low frequency EMS systems rely on sustainment or care deteriorates.

Based upon the needs identified in the previous paragraph, State licensing agencies employ systems and personnel to "inspect" education offerings, institutions and educational readiness (accreditation). Typically this is targeted toward suspected cases where recertification fraud occurs, student complaints are high and/or operational supervisors have complained about sustainment education.

We believe the US Army EMS program must monitor the above characteristics in its sustainment program in order to assure quality. Failure to do so can disrupt medical readiness.

Sincerely,

William E. Brown, Jr.
Executive Director

Cc: Col. Paul Mayer, Medical Director, US Army EMS Program



All organizations will submit a training site accreditation request to US Army EMS prior to conducting courses/training. Requests for training site accreditation must be initiated by the Commander. The following must be completed and/or submitted to US Army EMS prior to conducting training:

C-1. Request for Accreditation Site Code

1. Initiation of accreditation process
 - a. Commander's request memo (figure E-1) including:
 - i. Site location
 - ii. Courses/training to be conducted
 - iii. Commander's signature
2. Prior to first course
 - a. Site POC sheet (figure E-2) including:
 - i. Medical Control Officer
 - ii. Course Coordinator
 - iii. Primary Instructor
 - iv. Full-time POC or Operations POC
 - b. Key Personnel appointment memo (figure E-3)
 - i. Medical Control Officer
 - ii. Course Coordinator
 - iii. Primary Instructor
 - c. Signed equipment list (one for each individual course)
 - d. Instructor roster (figure E-4) including:
 - i. Name & rank
 - ii. PMOS/SMOS
 - iii. List of Credentials (MOS, certifications, licenses, related assignment/positions)
 - iv. NREMT card number & expiration date
 - v. Table VIII skills validation date

D-1. Request for Approved Course Number

Courses or training events conducted at the accredited training site must be individually approved by US Army EMS and assigned an Approved Course Number. Course Coordinators must complete and/or submit the following to US Army EMS no later than 30-days prior to conducting training:

1. Submit the following to US Army EMS Accreditation Branch:
 - a. Training Schedule for course(s)
 - i. Training location
 - ii. Approved site code & Organization
 - iii. Date/time
 - iv. Hours per lesson
 - v. Topic/Table
 - vi. Lesson reference(s)
 - vii. Instructor name
 - viii. Risk assessment level
2. Signed Memorandum of Agreement (MOA) for clinical rotations of 68W students in a federal organization and/or a Memorandum of Training (MTA) for clinical rotations of 68W students in a non-federal organization
3. Verify all equipment needed for course(s) is available and serviceable
4. Key Personnel and Instructor rosters must be current

D-2. Course Approval Process

Approved Course Number requests are processed by the US Army EMS Accreditation Branch. On receipt of a valid request, US Army EMS will:

1. Review and Validate Request
 - a. Confirm requesting site has valid Accreditation Site Code
 - b. Confirm requested course/training event is an approved course
 - i. Non-approved courses require "Crosswalk" (see section 2-03b)
2. Verify Site Accreditation
 - a. Review all accreditation documents and Site files (databases) for completeness
 - i. Review Instructor roster to verify credentials match course
 - ii. Verify POC information with CC
 - b. Identify any deficiencies in site files
 - i. Notify requesting unit by email of shortcomings and corrections needed
 - c. Review site visit / training observation records

The documents/records listed below will be maintained by the accredited training site and readily available for audit by US Army EMS for a minimum of two (2) years. Changes in personnel require updated accreditation documents. The minimum requirements for each document/record is indicated below:

- Current US Army EMS issued Memorandum of Accreditation
- Key personnel roster (MCO, CC, PI)
 - Name & rank
 - Credential
- POC form (phone and email)
- Instructors' credentials
 - MOS, certifications, licenses, related assignment/positions
- Training site SOP
 - Class quota/minimum requirements
Add Ratios
 - Classroom management plan
 - 68W MOS sustainment plan
 - o Where, when, how CEs and skills conducted and verified
 - Internal site audit / QA procedures
 - Records keeping procedures / requirements
 - Student/Course database procedures (MODS, US Army EMS Portal, etc.)
 - Rebuttal process
- Training schedules:
 - Approved site code & Organization
 - Training location
 - Date/time
 - Hours per lesson
 - Topic/Table
 - Lesson reference(s)
 - Instructor name
 - Risk assessment level
- Risk Management worksheets (use current DoD form)
- Equipment needed for course(s)

- Class roster and daily sign-in/out sheets
 - Date
 - Time in/out
 - Student Printed Name
 - Student Signatures (in and out)
 - Accreditation Site Code & Approved Course Number
 - Verifying Instructor (conducting class)
- Course completion document (DA7442, certificate, etc.)
 - Printed name and signature of training event Officer in Charge (OIC) or Noncommissioned Officer in charge (NCOIC)
 - Record date (on DA7442) that each individual training table (TC 8-800) was completed
- Course critiques and AARs

The Course equipment list can be found in the 68W Downloads section of [the 68W Combat Medic Website](#).

Appendix F

68W Recertification Requirements

1. Verify the 68W MODS Tracking System Individual Status Report (ISR) indicates: (1) current CPR certification, (2) at least 72 hours of CE and (3) Skills Validation within the current recertification period (past 2-years) (see figure F-1).

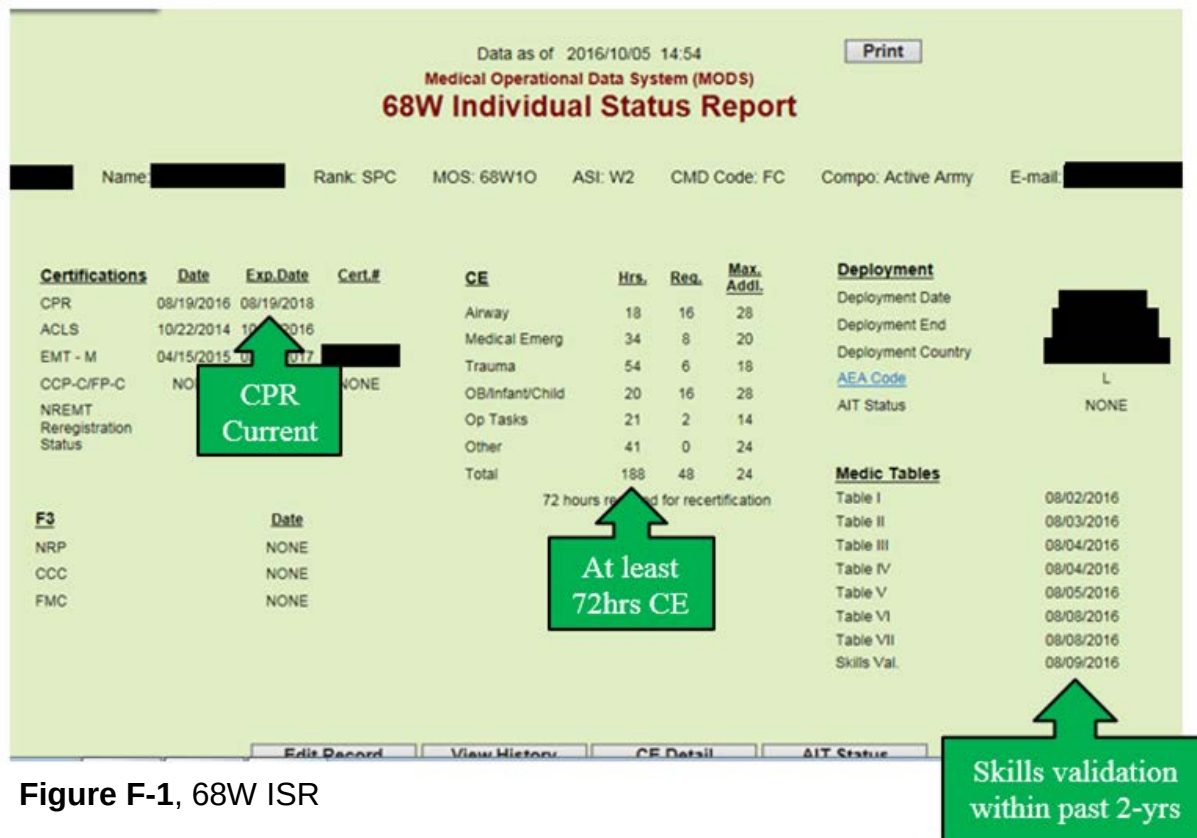


Figure F-1, 68W ISR



Figure F-2, 68W ISR

2. Missing or expired requirements are indicated by either red text or listed under "NREMT Reregistration Status" (see figure F-2). To update reregistration status, provide supporting/verification documents to your organization's 68W MODS writer before continuing to the NREMT site. If any requirements are missing, expired or incorrect in your ISR, MODS will not transfer data to the NREMT.
3. Once all requirements are complete and ISR displays the NREMT banner proceed to the NREMT website and initiate recertification process:
 - a. Log in to your account on the NREMT website
 - i. Ensure your "Agency" shows "Active" or "Reserves" or "National Guard" as appropriate. If unaffiliated, click "Affiliate with Agency" and select "Army" as the agency's state. Then select either Active, Reserve or National Guard (as appropriate) for your agency. Do not select a specific base or division or any other option.
 - b. Click "My Certification" on the left side of the screen
 - c. Click "Manage Education"
 - d. Verify that all hour and topic requirements are met for Refresher Training and Continuing Education
 - e. Verify your CPR certification is current
 - i. If no training or education information is displayed, verify the information is in the 68W Tracking System (see section F-1 above).
 - f. Complete the felony and disciplinary actions questionnaire
 - g. Click "Submit Recertification" at the bottom of the page

A completion page with "Thank you for submitting your recertification" will display after successful submission. Your application is electronically submitted to the NREMT for final approval. If the completion page does not appear, the recertification submission was not completed. Recertification status is displayed in the "My Certification" section of the NREMT site.

4. Any difficulties or issues with the NREMT online recertification process should be forwarded to the U.S. Army EMS Sustainment Branch.

Personnel with a SMOS of 68W must maintain certification with the NREMT in order to provide health care or perform 68W Combat Medic duties.

At minimum, the following requirement must be completed and submitted to US Army EMS Sustainment Branch prior to initiation of the recertification process:

- Complete the TC8-800, MEDIC training tables (I-VII) and skills validation table (VIII) at an Army EMS approved training site and provide course completion documents:
 - i. Training schedule
 - ii. Daily Instructor & Student sign-in/out sheets
 - iii. Course completion document (DA7442, training certificate, etc.)
- Additional 24 hours of continuing education
- Valid BLS Card
- NRPs must provide a signed memorandum of training by a Medical Director

Once all requirements are complete and approved by US Army EMS, proceed to the NREMT website and initiate recertification process:

1. Log in to your account on the NREMT website
 - a. Your affiliated agency must be “US Army EMS Programs Management”. If unaffiliated, contact the US Army EMS Sustainment Branch. Do not select a specific base or division or any other option.
2. Click “My Certification” on the left side of the screen
3. Click “Manage Education”
4. Verify that all hour and topic requirements are met for Refresher Training and Continuing Education
5. Verify your CPR certification is current
6. Complete the felony and disciplinary actions questionnaire
7. Click “Submit Recertification” at the bottom of the page
8. Pay the recertification fee.
9. Notify US Army EMS Sustainment Branch once all actions have been completed.

A completion page with “Thank you for submitting your recertification” will display after successful submission. Your application is electronically submitted to the NREMT for final approval. If the completion page does not appear, the recertification submission was not completed. Recertification status is displayed in the “My Certification” section of the NREMT site.

Any difficulties or issues with the NREMT online recertification process contact the Army EMS Sustainment Branch.

Overview

1. The (critical) Combat Medic tasks and skills selected for training and testing (in TC8-800) address the three leading causes of preventable death on the battlefield:
 - a. Hemorrhage,
 - b. Tension Pneumothorax
 - c. Airway Problems
2. The critical tasks are life-saving skills indispensable at the point of wounding. Combat Medics must be proficient in every critical task. Each individual critical tasks is grouped into Training Tables:
 - a. Table I - Trauma
 - b. Table II - Airway
 - c. Table III -Intravenous Access and Medication Administration
 - d. Table IV - Medical
 - e. Table V - Triage and Evacuation
 - f. Table VI - Force Health Protection
 - g. Table VII - Obstetrics, Gynecology and Pediatrics
3. TC8-800 specifies the critical Combat Medic tasks for each Training Table. However, these critical tasks are only the minimum requirements. Commanders may add additional tasks/skills to meet the Unit's METL, training cycle or operational environment. Additionally, skill level 2 & 3 tasks should be incorporate into the training cycle to ensure 68W NCOs remain proficient in the tasks at their skill level.

Course Approval

- Requests for TC8-800 course approval will be submitted to US Army EMS no later than 30-days prior to the projected start of the course. Requests will include a complete training schedule following the format approved by US Army EMS. At minimum, training schedules will contain:
 - a. A minimum of 48-training hours broken down into the seven training tables as listed in TC8-800
 - b. All critical tasks will be covered (and listed) in either lecture or practical exercises
 - c. Instructor rehearsal/preparations for skills validation testing
 - d. Skills validation familiarization and practice prior to skills validation testing
 - e. Skills validation orientation briefing
 - f. Table VIII skills validation is to be conducted on the last day. No other training is authorized on skills validation testing day.
 - g. Reference for each lecture and practical exercise block of instruction

- Instructors roster must be up-to-date. Instructors not listed on roster (guest instructors) must have credentials approved by US Army EMS.
- Proper Instructor to Student ratios will be maintained during training and testing to ensure safety and efficiency. Standard ratios (Instructor: Student) are as follows:
 - a. Classroom lecture: 1:24
 - b. Classroom practical exercise: 1:6
 - c. Invasive procedures: 1:1 (direct observation)
 - d. Skills validation testing: 1:1 (direct observation, per casualty)

Critical Tasks Proficiency

1. TC8-800 training will be a formal training and evaluation activity that is scheduled & structured. The training will focus on skills demonstration, practical exercise and demonstrated proficiency (using individual skills sheets) of critical tasks - conducted "lanes" style or in classroom.
2. Instructors explain and demonstrate each skills/task to standard. Students practice skills/tasks until proficient. Individual skills/tasks can be combined and performed concurrently. Once students demonstrate proficiency, Instructor evaluates task/skill to standard (utilizing appropriate DA Form 7595). Skills "check-off" conducted either during PE or performed in separate area with another instructor.

Skills Validation

1. The key to skills validation is the Combat Medic's demonstrated ability to perform hands-on life-saving tasks to standard. It is ultimately the Combat Medic's performance of these medical skills to prescribed standard that must be tested and validated.
2. Medical and trauma scenarios are used to validate critical skills in dynamic patient care scenario. These scenarios validate the Combat Medic's ability to apply skills in situations consistent with casualties in a tactical pre-hospital environment. Treatment of casualties in Skills Validation will include all subtasks in the scenario (multiple skills performed for each scenario).
3. Each Combat Medic will complete two medical and two trauma scenarios (total of four casualties).
4. Evaluation focuses on the Combat Medic's demonstrated ability to perform hands-on life-saving tasks to standard:
 - a. Assess, treat and stabilize two trauma casualties
 - b. Perform appropriate life-saving measures on two medical casualties
 - c. Triage, treat and evacuate the casualties

5. Skills validation skills practice will be incorporated into each TC8-800 training schedule to properly orientate and prepare students for testing. Table VIII skills validation testing will be conducted using only the 25 provided scenarios (15 trauma and 10 medical) found in Appendix A of TC8-800. The evaluators will utilize the scenario, the appropriate Trauma or Medical grade sheet (DA7741 or DA7742) and the applicable Task Grading Sheets (DA 7595-x-x) when evaluating each Combat Medic.

Skills Validation Requirements

1. Skills validation may be accomplished utilizing a number of methods: simulation lab, trauma lanes, TC3 lanes, MOUT site, training area, etc.. Skills validation requires two trauma scenarios and two medical scenarios.
2. The use of simulations and realistic scenarios to add variety, realism and interest is authorized. However, simulation must not detract from training and the testing of skills.
3. All skills validation stations require a live simulated casualty or a mannequin. Trauma moulage should be used as appropriate. Avoid excessive or overly dramatic use of moulage; it must not interfere with the Combat Medic's ability to treat the patient.
4. Because of moulage limitations, the evaluator must establish a dialogue and provide feedback to the Combat Medic. However, the Evaluator will not coach the Combat Medic, except to start or stop the evaluation.
5. All procedures, interventions and assessments must be performed on a simulator or another Soldier. There is no verbalization and/or notional skills validation.
6. The Combat Medic is required to accomplish all assessment steps listed on the graded scenario skill sheet.
7. The evaluation process consists of at least one evaluator for each casualty observing a single Combat Medic's performance. The evaluator manages the flow of the scenario by supplying information pertaining to sight, sound, touch, smell and injuries that cannot be realistically moulaged, but would be immediately evident in a real casualty. The evaluator must also observe and record the events on the skills grade sheet. The evaluator will record two times; real time and stop watch time.

Skills Validation Process

1. Combat Medics will receive an initial briefing/orientation and instructions in the skills validation procedures. A standardized orientation script (available in Chapter 3 of TC8-800) will be read aloud before each skills validation testing. Clear and concise directions must be given as to what is expected of them during the skills validation.
2. The same two medical and two trauma scenarios will be used for the entire skills validation day. Once a lane is established for skills validation, it must be used for all Combat Medics being testing. This ensures consistency of the evaluation. Evaluators will familiarize themselves with as much detail about the scenario as possible to minimize interruptions in the flow of the scenario.
3. Combat Medics must wait outside the testing area until a lane is open. The evaluator will call a Combat Medic to the lane when ready for testing.
4. One Combat Medic at a time is evaluated at each scenario station. A non-testing Combat LifeSaver assistant may be used to assist with non-evaluated tasks. The testing Combat Medic is responsible to ensure any improper actions taken by the assistant are corrected.
5. Students are not to be used as skills validation evaluators or buddy/assistants.
6. Students cannot be used as simulated patients or non-testing Combat LifeSaver assistant unless they have completed and passed all Table VIII requirements.
7. Retesting of a failed scenario station may be accomplished after immediate retraining or after a more formalized training session. A different scenario will be used.

AAR – After Actions Reviews

AEMT – Advanced Emergency Medical Technician

AI – Assistant Instructor

AIT – Advanced Individual Training

ALARACT – All Army Activities

AMEDD – Army Medical Department

AMEDDC&S, HRCoE – Army Medical Department Center and School, Health Readiness Center of Excellence

APPD – AMEDD Personnel Proponent Directorate

AR – Army Regulation

ARNG – Army National Guard

ASAP – US Army Substance Abuse Program

ASI – Additional Skill Identifier

ATRRS – Army Training Requirements and Resources System

AWOL – Absent Without Leave

CAC – Common Access Card

CBB – Criminal Background Branch

CBQ – Criminal Background Questionnaire

CC – Course Coordinator

CE – Continuing Education

CEU – Continuing Education Unit

CLS – Combat Life Saver

CMP – Course Management Plan

COMPO – Army Component

CONUS – Continental United States

COR – Contracting Officer Representative

CPR – Cardiopulmonary Resuscitation

DFR – Dropped From Rolls

DoD – Department of Defense

DPTMS – Directorate of Plans, Training, Mobilization and Security

EMS – Emergency Medical Services

EMT – Emergency Medical Technician

FAQ – Frequently Asked Question

FSR – Field Service Report

FY – Fiscal year

IET – Initial Entry Training

IMCOM – Installation Management Command

ISR – Individual Status Report

MACOM – Major Command

MCO – Medical Control Officer

MEDCEN – Medical Center

MEDCOM – US Army Medical Command

MEDDAC – Medical Department Activity

MEDIC – Medical Education and Demonstration of Individual Competence

MOA – Memorandum of Agreement

MODS – Medical Operational Data System

MOS – Military Occupational Specialty

MOS-Q – Military Occupation Specialty Qualified

MOS-T – Military Occupation Specialty Reclassification Course

MSTC – Medical Simulation Training Center

MTA – Medical Training Agreement

NCO – Noncommissioned Officer

NCOIC – Noncommissioned Officer in Charge

NREMT – National Registry of Emergency Medical Technicians

NRP – Nationally Registered Paramedic

OCONUS – Outside Continental United States

OIC – Officer in Charge

OIP – Organizational Inspection Program

OTSG – US Army Office of the Surgeon General

PEOSTRI – Program Executive Office for Simulation, Training and Instrumentation

PI – Primary Instructor

PM MedSim – Medical Simulation Program Manager

POC – Point of Contact

POI – Program of Instruction

POR – Program of Record

QA – Quality Assurance

QAR – Quality Assurance Representative

QC – Quality Control

RTI – Regional Training Institute

SAV – Staff Assistance Visits

SME – Subject Matter Expert

SMOS – Secondary Military Occupation Specialty

SOP – Standard Operating Procedures

STP - Soldier Training Publication

SRTP – Sustainment Readiness Training Program

SSN – Social Security Number

TC – Training Circular

TRADOC – Training and Doctrine Command

TSP – Training Support Package

UCMJ – Uniform Code of Military Justice

USAR – US Army Reserve